



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : بخيته عفيفى متياس شنوده

العنوان : البستان

الحي / المدينة : الغردقه

المحافظة : البحر الاحمر

تاريخ تسجيل الإستماره : 03-10-2026

رقم التليفون : 01222762118

رقم البطاقة : 27708292501867

العمر (تاريخ الميلاد) : 29-08-1977 المهنة : بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإخصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / ----- أ.د. / ----- أ.د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: يخته عفيفي متياس شندوه
National ID: 27708292501867



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

Name : Bakhita Afifi Matias Shenouda

Date : 25-06-2026 07:30

Branch : Dokki

Medical No. : 402156

Ref.Physician Prof : Goustav Roussey Egypt

WHOLE-BODY BONE SCINTIGRAPHY REPORT

Indication: Staging evaluation for a 48-year-old female patient with history of right breast carcinoma (post-surgical resection and chemotherapy; no radiotherapy). No specific bone pain reported.

Technique: Whole-body planar scintigraphy in anterior and posterior projections following intravenous administration of Tc-99m MDP.

Findings:

- **Axial Skeleton:** Uniform physiological uptake throughout the cervical, thoracic, and lumbar spine, sacrum, iliac wings, ribs, and sternum. No focal hot spots suggestive of osteoblastic metastasis.
- **Appendicular Skeleton:** Symmetrical physiological activity in humeral/femoral heads, glenohumeral and sacroiliac joints, and cortical long bones of all extremities. No focal anomalies.
- **Soft Tissue & Excretion:** Normal renal excretion with physiological bladder activity. No abnormal extraosseous tracer retention.

Conclusion:

- Negative whole-body bone scan with no scintigraphic evidence of osteoblastic metastatic bone disease. Findings represent normal physiological radiotracer distribution.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

جمهورية مصر العربية
 بطاقة تحقيق الشخصية

بخيته
 عفيفي ميثاق شنودة
 البستان - السكن الإداري
 شان الغردقة - البحر الأحمر

٢٧٧.٨٢٩٢٥.١٨٦٧



KI8261005

٢٠٢٣/١٢ ٢٧٧.٨٢٩٢٥.١٨٦٧
 ربة منزل

متزوجة
 مسيحية
 أمش
 عماد نظمي فوزي عجيان
 ٢٠٣٠/١٢/٢٥
 البطاقة سارية حتى

١٤

د. م. س.
 فسيح

Name: بخينه عفيفى منياس شنوده		
MR. : 9398		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 49-year-old individual who is pre-menopausal. The clinical history is notable for a prior right-breast cancer diagnosed on 3/1/2026, described as high-grade ductal carcinoma in situ (DCIS) with microinvasion, pathologic stage pT1 invasive ductal carcinoma (IDC) with 2 positive nodes out of 7 (pN1/7). The tumor was estrogen-receptor (ER) 8/8, progesterone-receptor (PR) 5/8, HER2 3+ (positive), with a recurrence score of 4 and an anatomic classification of 7 weeks. The patient received 4 weeks of taxol, after which the neoplastic tissue was noted to abut the inferior and medial borders. Subsequently, the patient completed 12 weeks of taxol (4 cycles), taxotere, carboplatin, and Phesgo, with the last administration on 12/9/2026. Smoking history is 1/8 pack-years, currently free. Biopsy of the right breast confirmed high-grade DCIS with microinvasion and the biomarker profile described above (ER 8/8, PR 5/8, HER2 3+). Imaging, including sonomammography, demonstrated residual disease abutting the inferior and medial borders of the breast tissue. No additional metastatic workup findings were documented. The referring team recommended surgical consultation for completion mastectomy. The multidisciplinary team concurred with the recommendation and plans to proceed with completion mastectomy as the definitive surgical management.		
تحريرا في ,		
	الوقت :	التاريخ : 23-09-2026
		الطبيب المعالج :
يعتمد ,		

MDT FORM

Name:	بختة عفيفي متياس شنوده	MRN:	9398
Age:	49 Y	ID:	27708292501867
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: 3/8 **PR:** 0/8 **HER2:** ampli **KI67:**

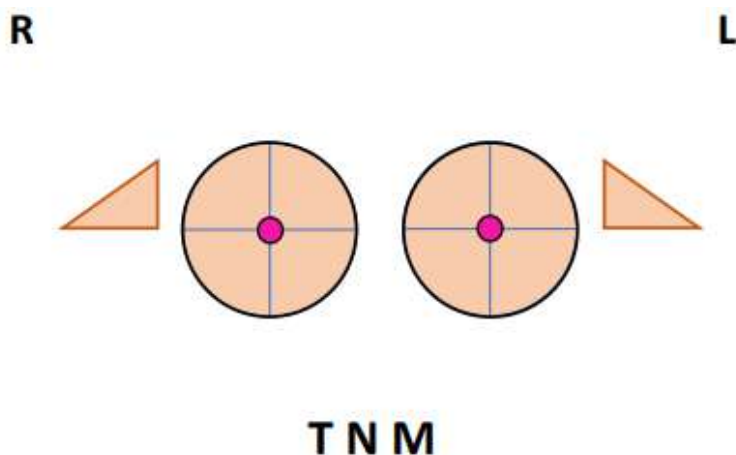
Sonomammography:

Mets Workup:

History:
A CASE RIGHT BREST CANCER BCS T 3 5X4 CM N1 1/7 MO ER 3/8 PR 0/8 HER +2 AMPLIFIED RECIEVED 3 /4 Ac in GHardaa referred for antiher2

Recommendations:

MDT Outcome & Plan
PT UNDERWENT CLIPPING CONTINUE NACT TAXOL weekly PHSEGO x 4 then WLE+SLND



SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد الكيلاني د/ ندي حسن		د/ نرمين رمضان			

Patient Name :	الأستاذة / بختياره عفيفى متياس	Request No. :	13005346
Patient No. :	27708292501867	Request Date :	08/03/2026
Path No. :	SR-344/26	Verification Date :	11/03/2026
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	48 Y	Department :	Out Patient
Referreing Doc :		Registered by :	.marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer.

Nature of specimen: Referred blocks & slides (Right wide local excision & Sentinel lymph nodes biopsy).

Gross Examination:

- Nine referred blocks labelled by (64942) & nine referred slides labelled by (65939) sublabelled from (1:9).

Microscopic Examination:

- * Histological type: Microinvasive breast carcinoma (Slide 5).
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * DCIS: Extensive high grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Surgical margins: Refer to the original report.
- * Sentinel lymph nodes: Positive metastatic deposits in one out of seven lymph nodes (1/7) with no extra-nodal extension.

Diagnosis:

Right breast cancer, Referred block & slides (Right breast wide local excision & Sentinel lymph nodes):

- Microinvasive breast carcinoma in a background of extensive high grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
- Sentinel lymph nodes: Positive metastatic deposits in one out of seven lymph nodes (1/7) with no extra-nodal extension.
- * Stage: pTmi, pN1a (sln).
- ** Immunohistochemistry is in another report.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذة / بختة عفيفي متياس

Patient No. : 27708292501867

Path No. : SR-344/26

Sex : Female

Age : 48 Y

Referring Doc :

Request No. : 13005346

Request Date : 08/03/2026

Verification Date : 11/03/2026

Financial Category : جوسف روسي - نفقة الدولة

Department : Out Patient

Registered by : .marwa mohamed youss

Pathology Report

Signature

Prof. Dr. Maha Guimei.

Verified by :

Dr.Duaa Mohamed

Printing Date : 20/05/2026 10:41:55

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / بختياره عفيفي متياس	Request No.	: 13005346
Patient No.	: 27708292501867	Request Date	: 08/03/2026
Path No.	: SR-344/26	Verification Date	: 17/03/2026
Sex	: Female	Financial Category	: جوسفان روسي - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer diagnosed "Microinvasive breast carcinoma in a background of extensive high grade DCIS, with positive one lymph node" stage; pTmi, pN1a (slns).
Nature of specimen: Referred blocks & slides (Right breast wide local excision & Sentinel lymph nodes biopsy).

Gross Examination:

- Nine referred blocks labelled by (64942) & nine referred slides labelled by (65939) sublabelled from (1:9).

Technique:

- Sections prepared from paraffin blocks (2 & 5) (Lymph node & microinvasive) for immune-staining using Bench Mark ULTRA system for;
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (Positive control showed appropriate reactivity) (Done on the positive lymph node & microinvasive component):
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
 - Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Conclusion:

Right breast mass, Referred blocks & slides (Right breast wide local excision & Sentinel lymph nodes biopsy):

IPT:

- ER: Positive (score 8/8).
 - PR: Positive (score 5/8).

Verified by :

Dr.Duaa Mohamed



Patient Name	الأستاذة / بختة عفيفي متياس	Request No.	13005346
Patient No.	27708292501867	Request Date	08/03/2026
Path No.	SR-344/26	Verification Date	17/03/2026
Sex	Female	Financial Category	جوسف روسي - نفقة الدولة
Age	48 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- Her2: Positive (score 3+).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 20/05/2026 10:42:04

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Mrs. Bekheta Afefy

Date: 25 October 2025

Referred from: Prof. Dr.

BREAST ULTRASONOGRAPHY

MORPHOLOGY:

** RIGHT BREAST:

- Shows mild dilatation of the main ducts with smooth regular wall, no detected definite intraductal masses or other abnormalities, the dilated ducts of average 3.5mm in diameters, almost of equal sizes. No retero-ariolar lesions.
- Prominent fatty glandular tissue no detected abnormalities.
- No detected solid masses.
- No micro or macrocalcification.
- Intact skin with no skin thickening.
- Normal appearance of the axillary tail on both sides.

** LEFT BREAST:

- Normal fatty glandular tissue no detected abnormalities.
- Intact ducts, no abnormal ductal dilatation. No retention cysts.
- No detected solid masses.
- No micro or macrocalcification.
- Intact skin with no skin thickening.
- Normal appearance of the axillary tail on both sides.

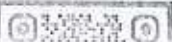
** BOTH AXILLAE:

Shows few mildly enlarged lymph nodes range from 10 to 13 mm in diameters with preserved hilum and oval shape .

CONCLUSION:

Right Breast Duct Ectazea, Of Inflammatory Nature, No Deetected Intraductal Masses Or Suspecious Masses with bilateral axillary lymphadenitis

MUCH OBLIGED
Dr. Mina Fallah Younan.





IMMUNOHISTOCHEMISTRY

Patient Name بخيتة عذيفي

Referring Doct

Branch:

معمل مستشفى رويال الفردقه د. محمد خاـ Sex Fer Age 47 Y

Lab. No. : 6001909967

Reg. Date: 04-01-2026 09:08

Internal Cod 3004811

Nature of Specimen: Paraffin block

GROSS:

Immunohistochemistry for ER, PR, Her2 & KI67.

MICROSCOPY:

- ER: (In the few invaisve foci) stain in 1/3 stain den 2/5 total socre 3/8 (weak).
- PR: (In the few invasive foci) Negative.
- Her2: (In the few invasive foci, positive (score 2+) to be confirmed by (ISH).
- KI67: About 30% in the few invasive foci.
- *ER & Her2 are positive in the insitu component.

DIAGNOSIS:

- IMMUNOSTAINING WAS ASSESED IN THE FEW INVASIVE FOCI REVEALED:
- ER:TOTAL SOCRE 3/8 (WEAK).
- PR: NEGATIVE.
- HER2: POSITIVE (SCORE 2+) TO BE CONFIRMED BY (ISH).
- KI67: ABOUT 30%.
- *ER & HER2 ARE POSITIVE IN THE IN SITU COMPONENT.

Test were carried out on the VENTANA fully automated stainer (Discovery XT)

Signature

أحمد

أ.د / أماني عبيد الباري
استاذ جاثولوجي
كلية الطب - جامعة الاسكندرية

10615



HISTOPATHOLOGY REPORT

Patient Name	بختة عفيفي	Lab. No. :	6001899975
Referring Doct		Reg. Date:	03-01-2026 09:13
Branch:	معمل مستشفى رويال الغردقة د. محمد حنا	Sex	Female
		Age	47 Y
		Internal Cod	65939

Nature of Specimen:

Right WLE + SLNB

Gross Pathology:

Received are the following:

- *Sentinal L.N biopsy: 4x4x3 cm fibrofatty tissue fragment from which 7 LNs were retrieved [1/2].
- *WLE: 7x6x4 cm fibrofatty tissue fragment. C.S revealed 5x3x2 cm firm whitish mass abutting both medial and inferior margins grossly. The surgical margins were labelled as follows: [3] inferior/ [4] superficial/ [5] superior/ [6] lateral/ [7] medial/ [8/9] depth.

Histopathology Report:

CAP protocol for examination of specimens from patients with breast carcinoma.

- Procedure: WLE + SLNB.
- Laterality: Right.
- Tumor site: Not specified.
- Histologic type: Invasive carcinoma of no special type (ductal).
- Histologic grade: Nottingham histologic score; grade 2 (2, 2, 2).
- Tumor size: 5x3x2 cm.
- Tumor focality: Single focus.
- Ductal carcinoma in situ: Present and prominent.
- Size of DCIS: Most of the examined sections.
- Architectural patterns of DCIS: Solid and comedo.
- Nuclear grade of DCIS: Grade III.
- Necrosis: Present, focal (small foci or single cell necrosis).
- Lymphatic and vascular invasion: Present.
- Margin status: The neoplastic growth is seen abutting the inferior and medial margins while the rest are free with the narrowest foci [0.1-0.2 cm] seen within the superior margin and depth.
- Lymph node status: One out of the seven submitted dissected lymph nodes is invaded without nodal capsular or extra nodal fat infiltration (1/7).

Conclusion:

- *RIGHT WLE + SLNB:
- RESIDUAL FOCI OF HIGH GRADE DCIS (COMEDO AND SOLID PATTERN) ASSOCIATED WITH FOCI OF MICROINVASIVE DUCTAL CARCINOMA G II.
- POSITIVE VASCULAR INVASION.
- THE NEOPLASTIC GROWTH IS SEEN ABUTTING THE INFERIOR AND MEDIAL BORDERS.
- THE REST OF THE BORDERS ARE FREE WITH THE NARROWEST FOCI RANGING FROM 0.1-0.2 CM SEEN WITHIN THE SUPERIOR MARGIN AND DEPTH.

أ.د. / رضوي محمد رشاد
استاذ مساعد طب باورجى
معهد البحوث الطبية



-ONE OUT OF THE SEVEN SUBMITTED SENTINEL LN SHOWS METASTATIC DEPOSITS WITH
NEITHER CAPSULAR NOR EXTRA NODAL FAT DEPOSITS.

Notes:

* يتم الاحتفاظ بالعينة لمدة شهر واحد من تاريخ استلامها، كما يتم الاحتفاظ ببلوكات الشمع لمدة ثلاث سنوات كاملة

أ.د / رضوى محمد رشاد
أستاذة مساعد الأوباثولوجي
مركز الأبحاث الطبية



Patient Name: Bekheeta Afifi Metyas Shenouda

Referred By: -

Branch: El Asafra

Acc No.: 100572601

Reg. Date: 22-01-2026 11:32

Sex: F Age: 49 Y 0 M Rep. Date: 23-02-2026 11:13

FISH REPORT

Primary sample: Breast FFPE tissue.

Probe specifications: Sections prepared from the paraffin block were stained for HER 2 gene and chromosome 17 by Fluorescence in Situ Hybridization (FISH) dual probe technique (PathVysion).

Result: The ratio of Her 2 signals to chromosome 17 signals in the nuclei of the tumor cells is 2.4 (>2) indicating amplification of HER2 gene.

Cutoff value: Positive: FISH HER2 CEP17 ratio ≥ 2 .
FISH HER2 gene copy number ≥ 4 .
Negative: FISH HER2 CEP17 ratio < 2
FISH HER2 gene copy number < 4 .

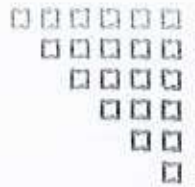
Reference:

Wolff AC, Hammond MEH, Allison KH, Harvey BE, Mangu PB, Bartlett JMS, Bilous M, Ellis IO, Fitzgibbons P, Hanna W, Jenkins RB, Press MF, Spears PA, Vance GH, Viale G, McShane LM, Dowsett M. Human Epidermal Growth Factor Receptor 2 Testing in Breast Cancer: American Society of Clinical Oncology/College of American Pathologists Clinical Practice Guideline Focused Update. J Clin Oncol. 2018 Jul 10;36(20):2105-2122.

Conclusion: The case is Positive for HER-2/neu gene amplification.

19675

أ.د / ريهام عبد الحليم
استاذ الباثولوجيا الإكلينيكية
كلية الطب - جامعة الاسكندرية



مستشفى رويال
ROYAL HOSPITAL
HISTOPATHOLOGY REPORT

بجينة عفيفي مرياس
F-47Y 1:43AM 18-12-25



3 025121 910042
18-December-2025 1:43 AM

Patient Name بجينة عفيفي

Referring Doctor

Branch: معمل مستشفى رويال الفرقة د. محمد حنا Sex Fer Age 47 Y

Lab. No. : 6001790211

Reg. Date: 19-12-2025 11:07

Internal Cod: 64942

Nature of Specimen: Right breast mass excision biopsy

Gross Pathology:

Received is a 5x4x2 cm fibrofatty tissue specimens labelled by threads and showing a 4x3.5x2 cm whitish areas. Separate 5x4x1.2 cm fragmented fibrofatty tissue pieces are received.

Histopathology Report:

Microscopic examination reveals dilated ducts with mostly sloughed lining, formed of malignant cells having increased N/C ratio, and large pleomorphic hyperchromatic nuclei, with prominent nucleoli. The chromatin is coarsely-clumped. The cytoplasm is abundant and acidophilic groups of malignant cells are seen infiltrating the stroma. Focal necrosis is noted within many ducts forming comedo pattern as well as solid pattern. Positive lymphovascular invasion is seen. The growth is seen abutting the superior margins. The separate fragments are invaded by the same growth.

Conclusion:

HIGH GRADE INSITU DUCTAL CARCINOMA, WITH FOCI OF STROMAL INVASION.
-THE TUMOUR IS SEEN ABUTTING THE SUPERIOR MARGIN, THE OTHER MARGINS ARE FREE.
-THE SEPARATE TISSUE FRAGMENTS ARE INVADED BY THE SOME TUMOUR.
-POSITIVE LYMPHOVASCULAR INVASION IS SEEN.

Notes:

I.H.C FOR ER, PR, HER-2 AND KI-67 IS RECOMMEDNED AND AVAILABLE UPON

نهر واحد من تاريخ استلامها، كما يتم الاحتفاظ ببلوكات الشمع لمدة ثلاث سنوات كاملة



C172



أدلة التوجيه
استاذة التوليد
د. د. د.



0653547662 - 67788 - 01271006200



HISTOPATHOLOGY REPORT

Patient Name	يحيى عتيق	Lab. No. :	6001790211
Referring Doct		Reg. Date:	19-12-2025 11:07
Branch:	معمل مستشفى رويال الفردقه د. محمد حاء	Sex	Fe
		Age	47 Y
		Internal Cod	64942

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Right breast mass excision biopsy

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Microscopic examination reveals dilated ducts with mostly sloughed lining, formed of malignant cells having increased N/C ratio, and large pleomorphic hyperchromatic nuclei, with prominent nucleoli. The chromatin is coarsely-clumped. The cytoplasm is abundant and acidophilic groups of malignant cells are seen infiltrating the stroma. Focal necrosis is noted within many ducts forming comedo pattern as well as solid pattern. Positive lymphovascular invasion is seen. The growth is seen abutting the superior margins. The separate fragments are invaded by the same growth.

Conclusion:

HIGH GRADE INSITU DUCTAL CARCINOMA, WITH FOCI OF STROMAL INVASION.
-THE TUMOUR IS SEEN ABUTTING THE SUPERIOR MARGIN, THE OTHER MARGINS ARE FREE.
-THE SEPARATE TISSUE FRAGMENTS ARE INVADED BY THE SOME TUMOUR.
-POSITIVE LYMPHOVASCULAR INVASION IS SEEN.

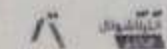
Notes:

I.H.C FOR ER, PR, HER-2 AND KI-67 IS RECOMMEDNED AND AVAILABLE UPON REQUEST.

* وتم الاحتفاظ بالعينة لمدة شهر واحد من تاريخ استلامها، كما يتم الاحتفاظ ببلوكات الشمع لمدة ثلاث سنوات كاملة

أ.د/ الشاذلي إبراهيم السرحة
استاذ علم الأمراض
جامعة الإسكندرية





Patient's Name: bekhet afefy abas shenoda
Date: 2026-04-05
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

**A known case of managed right breast cancer ,no available clinical data or
previous examination**

I- CHEST:

- Left lower lobar atelectatic band is noted
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (splenule measures about 1.5x1.1 cm), pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticular or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- No pulmonary or hepatic deposits.
- Hepatomegaly for laboratory correlation

Dr.Norhan Ramzy,MD